BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show order, not equal periods. These are expected checks, not booked appointments. Record the booking owner, due window, result reviewer and escalation contact.

Check	Before starting	First cycle / visits	Early reassessment	Later cycles	Change / completion
Bloods / symptoms	FBC, renal and liver profile	FBC, renal and liver before each cycle	After two cycles (~week 6): assess response	Repeat checks before treatment; extra tests if needed	Follow abnormalities; agree next care
ECG / electrolytes	Review cardiac history, medicines and QT risk	ECG if clinically indicated	Review symptoms and new medicines	Repeat according to risk / results	Agree follow-up if abnormal
Echo / MUGA	Echo / MUGA if cardiac history or prior anthracycline	Repeat if clinically indicated	New cardiac symptoms: reassess promptly	No fixed routine 3-month echo for this medicine alone	Exposure- and risk-based cardiac follow-up
Imaging / markers	Baseline disease assessment	Earlier review for new symptoms	Response review after two cycles (~6 weeks)	Breast: generally every 2–4 months; individualise	Review scans with symptoms; markers are supplementary
Bone health / DEXA	Assess endocrine / fracture risks	No automatic cycle-based DEXA	DEXA if indicated	Repeat according to bone-risk plan	Agree ongoing bone care

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00230 v7 · official source · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026 · EMA Abraxane product information

THE WHOLE TREATMENT JOURNEY

Nab-paclitaxel · every 3 weeks: the treatment pathway

Metastatic breast cancer after first-line treatment

Nab-paclitaxel on Day 1 every 21 days. Review response after two cycles; the NCCP protocol says to discontinue if there is no response. Further treatment depends on benefit and tolerance.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / visits	Early reassessment	Later cycles	Change / completion
Cancer medicine NCCP 00230	Confirm exact indication / formulation	Treat on Days 1 of 21	Response review after two cycles	Continue / adjust / pause after review	Agree next treatment or care
Surgery / radiotherapy METASTATIC	Define any local-treatment goal	No routine concurrent radiation assumed	MDT agrees whether to sequence or overlap	Agree any chemotherapy pause with both teams	Reassess symptom / local-control needs
Bloods / ECG NCCP 00230 · SMPC	Baseline bloods / cardiac assessment	Bloods before each cycle	ECG per risk	Review results before treatment	Follow abnormalities
Echo / MUGA NCCP 00230 · SMPC	Risk-based baseline echo / MUGA	Earlier test for cardiac symptoms	Review exposure / cardiac risk	Repeat when clinically indicated	Agree cardiac follow-up
Imaging / markers NCCP 00230 · METASTATIC · MARKERS	Baseline disease assessment	Symptoms may bring scans forward	After two cycles (~6 weeks)	Review scans with symptoms / markers	Document benefit and next plan
Bone / additional care BONE	Check fracture and endocrine risks	No automatic maintenance phase	DEXA if indicated	Supportive and palliative care as needed	Agree ongoing care

What each check is for

1 **Bloods**

FBC, renal and liver profile before starting and before each 21-day cycle. Review numbness, tingling and daily function before treatment.

Results and symptoms guide treatment safety.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00230

2 **Heart checks**

Baseline cardiac assessment, which may include ECG, echo or MUGA, if significant cardiac history or previous anthracycline therapy. Repeat cardiac assessment if clinically indicated. Baseline echo / MUGA when cardiac history or previous anthracycline treatment makes it necessary; ongoing testing is driven by symptoms and risk, with no universal 3-month schedule for nab-paclitaxel alone.

ECG and echo answer different questions.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00230 · SMPC

3 **Response review**

After two cycles, around week 6: assess response before proceeding. The source says to discontinue if no response; the team decides which examinations or scans establish response. This earlier source-specific review takes priority over the general 2–4-month metastatic breast interval.

Review benefit before continuing treatment.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00230 · METASTATIC

4 **Handover**

At initiation, each review and treatment change

Name the booking owner, due window, reviewer and escalation contact.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00230

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-18 · Source reconciled; independent clinical, pharmacy and patient review pending · SACTCheck v0.84.1